



CLINICAL PRACTICE GUIDELINE

Eating Disorders in Pregnancy

This document should be read in conjunction with the [Disclaimer](#)

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Aim

The purpose of this guideline is to identify patients with eating disorders in pregnancy, ensure referral is made to the appropriate antenatal clinic and that care aligns with referral criteria and principles of management. This guideline is consistent with WA Eating Disorders Outreach and Consultation Service (WAEDOCS) guideline "[Eating Disorders: the Management of Youth and Adults - A Quick Reference Guide](#)".

Keypoints

- Eating disorders (anorexia nervosa, bulimia nervosa and binge eating disorder) can be described as an extreme disturbance in behaviours that involve food restriction, binge eating and purging.
- Eating disorders are increasingly understood as complex neurobiological disorders, often precipitated by changes in weight and nutrition triggering brain-related changes which further drive the eating disorder cognitions and behaviours.
- Eating disorder in pregnancy is not a common occurrence with only a few population based studies providing prevalence data:
 - A UK study identified 0.05% prevalence of anorexia nervosa and 0.04% bulimia nervosa at 12 weeks gestation.

- The Norwegian Mother and Child cohort study suggests an overall point prevalence of 7.5% at 12 weeks gestation. This comprises 0.5% anorexia nervosa, 0.1% bulimia nervosa, 1.8% binge eating disorder, 0.1% eating disorder not otherwise specified purging type and 5% other eating disorder not otherwise specified.
- There is an increased risk in the perinatal period for reactivation of prior eating disorder vulnerability, so it is important to screen for past history of an eating disorder at any age.
- Despite the low prevalence, eating disorder increases the risk for poorer health outcomes for pregnant women and their babies as well as birth complications.
- Safe restoration of nutrition is therefore critical for both medical and mental health recovery. Early involvement of psychiatry and dietetics is therefore recommended for perinatal services.
- If there is significant risk to the health and safety, or significant risk of harm, to the person clinicians should consider use of the [Mental Health Act 2014](#) providing there is evidence of the patient's incapacity to make treatment decisions; and there is no less restrictive way to provide the treatment the person needs.

Assessment

A comprehensive assessment by the referrer should include:

- Current body mass index (BMI) – formally measured weight and height (ideally including a pre-pregnancy measure as well as ongoing weight in pregnancy)
- Recent nutritional intake – last two weeks
- History of presenting illness
- Past mental health history
- Cognitive features
- Co-morbid conditions – medical, psychiatric including suicidal ideation or deliberate self-harm
- Family functioning
- Physical health assessment

Referral Criteria

- Women with a long standing history of, and /or active and current restrictive eating disorder (anorexia nervosa).
- BMI ≤ 15 – automatic acceptance and will be managed by Gold Team.
- BMI >15 to 17 – will be reviewed by Obstetric Medicine and Psychiatry and considered on a case by case basis. Patients will either be referred to Gold Team or back to original referrer.
- Other eating disorder referrals (e.g. bulimia nervosa and purging disorder) will be considered on a case by case basis by Obstetric Medicine and Psychiatry

where there is risk of pregnancy complications and antenatal care due to excessive purging.

- Further information for consideration in referral includes:
 - Physical health assessment – rapid weight loss, low weight, purging, blood pressure (<90mmHg systolic), heart rate (<40 bpm or >120bpm), Electrocardiogram for arrhythmia, blood screen (blood sugar, sodium, potassium, magnesium, phosphate, albumin, liver enzymes, neutrophils), temperature (<35.5 degrees).
 - BMI should never be the sole determinant for referral particularly in pregnancy where weight gain will occur as pregnancy progresses. There needs also to be consideration of gestation and expected weight gain, failure to gain weight in addition to weight loss. There should also be consideration of the cognitive features of eating disorders, such as distorted body image and fear of weight gain.
 - Psychiatric concerns – significant psychiatric risk such as deliberate self-harm or suicidal ideation. Moderate-high agitation and/or distress.
 - Maternal-fetal/infant attachment, bonding and infant safety.

Principles of Management

- To facilitate recovery, care plans must be developed in collaboration with the patient, and involvement of partner/close family member/personal support person where possible, particularly for discharge planning/relapse prevention. It is part of the continuum of care that commences at the time of admission.
- Multidisciplinary team approach inclusive of obstetrics, midwifery, mental health (psychiatry and psychology), dietetics and obstetric medicine - to meet as soon as possible.
- If patient requires admission for care to KEMH:
 - For mental health assessment and treatment – admission to Mother Baby Unit
 - For re-feeding (usually via nasogastric feeding) and medical monitoring – admission to KEMH ward (with 1 to 1 nursing to be considered and organised if required)
 - Dietetic, Mental Health, Physician, Midwifery and Obstetric 'team' care throughout admission
 - Devise individual re-feeding and monitoring plan by the team with minimum weekly multidisciplinary team reviews of this plan
- Regular monitoring of fetal growth (active restrictive eating behaviours).
- Regular monitoring of electrolytes (active purging behaviours).
- Monitoring and assessment for cardiac complications particularly prior to delivery.
- Individualised behaviour management plan.
- Discharge planning aimed at maintaining weight gain, normalising eating,

improving nutrition, and maintaining behaviour management.

References

- Grove, K., Langdon, F., Senaratne, S., Dickinson, J., and Galbally, M. (2018) Case report: Antenatal management of anorexia nervosa. *O&G Magazine*, 20(3) 66-68.
- Micali, N., Treasure, J., and Simonoff, E. (2007) Eating disorders symptoms in pregnancy: A longitudinal study of women with recent and past eating disorders and obesity. *Journal of Psychosomatic Research* 63(3) 297-303.
- Watson, H. J., Zerwas, S., Torgersen, L., Gustavson, K., Diemer E., Knudsen G.P., Reichborn-Kjennerud, T., and Bulik, C.M. (2017) Maternal eating disorders and perinatal outcomes: A three generational study in the Norwegian mother and child cohort study. *Journal of Abnormal Psychology* 126(5) pp 552-564.
- Watson, H., Torgersen, L., Zerwas, S., Reichborn-Kjennerud, T., Knoph, C., Stoltenberg, C., Meltzer, H. M., Ferguson, E., Haugen, M., Magnus, P., Kuhns, R., and Bulik, C. (2014) Eating disorders, pregnancy and the postpartum period: Findings for the Norwegian mother and child cohort study. *Norwegian Journal of Epidemiology* 24 (1-2) pp 51-62.
- WA Eating Disorders Outreach and Consultation Services (WAEDOCS) (2018) Eating disorders: The management of youth and adults – a quick reference guide. North Metropolitan Health Service.

Related policies

[“Eating Disorders: the Management of Youth and Adults - A Quick Reference Guide”](#) WAEDOCS, North Metropolitan Health Service

South Metropolitan Health Service – [Multidisciplinary team review process for persons presenting with eating disorders \(acute\) guidelines](#)

Related WNHS policies, procedures and guidelines

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